

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°141/MVE-BDBV/01/10/2026

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HIGHLIGHTS (last 24 hours)

A total of 66 new confirmed cases of Ebola Virus Disease (EVD), including 22 community deaths, have been reported. These cases originated in the provinces of North Kivu (33 cases), Ituri (28 cases), Haut-Uélé (3 cases), and Tshopo (2 cases). In addition, 16 deaths were recorded in Ebola Treatment Centers (ETCs). A new health zone has been affected: Alimbongo (North Kivu), with 4 confirmed cases, including 2 deaths.

323 new samples were analyzed; 66 of these samples were found to be positive for the Bundibugyo virus, representing a positivity rate of 20.4%.

ACCUMULATIVE
CASE
8,442

ACCUMULATIVE
DEATH
CONFIRMED
4,080

LETHALITY
48.3%

PATIENTS IN
ISOLATION/CTE
846

ACCUMULATIVE
HEALED
2,213

TRACKING RATES
CONTACTS
(Of the Day)
73.1%



Affected provinces and affected health zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



64 out of 167
health zones affected
(38.3%)

Summary of key points from the last 24 hours

For the day of October 2, 66 new confirmed cases of Ebola Virus Disease (EVD) including 22 community deaths were reported in 4 provinces: North Kivu (33 cases), Ituri (28 cases), Haut-Uélé (3 cases) and Tshopo (2 cases).

In addition, 16 deaths were recorded in the CTEs (10 in Ituri, 5 in North Kivu and 1 in Haut-Uélé), bringing the total number of confirmed deaths for the period under consideration to 38.

However, 31 patients (14 in North Kivu, 9 in Ituri and 8 in Haut-Uélé) were declared cured after obtaining two negative results for Ebola virus disease.

As of October 2, 2026, the DRC has recorded 8,442 confirmed cases and 4,080 deaths. (Case fatality rate: 48.3 %). Ituri remains the epicenter of the epidemic with 75.5% of cumulative confirmed cases reported and 42.4% of new confirmed cases. In contrast, North Kivu province has a case fatality rate higher than the national average, at 59.1%.

With the addition of the Alimbongo health zone in North Kivu, 64 out of 167 health zones (38.3%), spread across seven provinces, have been affected since the start of the epidemic.

Table 1. Distribution of confirmed cases and deaths by affected province, as of October 2, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	28	6370	2,933	46.0%	28/36 (77.8%)
North Kivu	33	1657	979	59.1%	17/34 (50.0%)
Haut-Uélé	3	352	146	41.5%	7/13 (53.8%)
Tshopo	2	48	16	33.3%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	10	4	40.0%	3/11 (27.3%)
South Ubangi	0	2	1	50.0%	1/16 (6.3%)
Total	66	8,442	4,080	48.3%	64/167 (38.3%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of October 2, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6370	2933	46.0%	28	5	10	15
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	203	46	22.7%	1	0	0	0
Boga	2	2	100.0%				0
Bunia	1759	574	32.6%	5	0	2	2
Damascus	40	9	22.5%				0
Drodoro	15	7	46.7%				0
Fataki	82	31	37.8%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	37	12	32.4%				0
Komanda	173	106	61.3%	3	0	2	2
Lita	289	145	50.2%	1	0	0	0
Logo	13	5	38.5%				0
Lolwa	34	9	26.5%				0
Mahagi	4	1	25.0%				0
Mambasa	32	12	37.5%				0
Mandima	156	79	50.6%	4	0	0	0
Mangala	362	156	43.1%				0
Mongbwalu	706	333	47.2%	1	0	0	0
Nia-Nia	309	173	56.0%	4	3	3	6

Nizi	784	281	35.8%	1		1	1
Nyankunde	130	37	28.5%	1	0	0	0
Rimba	12	4	33.3%				0
Rwampara	1104	376	34.1%	7	2	2	4
Tchomia	76	25	32.9%				0
To ventilate	N / A	493	N / A				0
North Kivu	1657	979	59.1%	33	16	5	21
Blessed	376	252	67.0%	9	5	4	9
Good	19	7	36.8%				0
Butembo	272	174	64.0%	2	1	0	1
Goma	1	0	0.0%				0
Kalunguta	59	21	35.6%	2	2	0	2
Katwa	579	362	62.5%	8	3	0	3
Kayna	3	2	66.7%	2	2	0	2
Kyondo	34	20	58.8%	1			0
Lubero	22	11	50.0%	1			0
Mabalako	41	15	36.6%	3	1	0	1
Alimbongo	4	2	50.0%	4	2	0	2
Manguredjipa	13	5	38.5%				0
Masereka	31	11	35.5%	0	0	1	1
Musienene	127	68	53.5%	1	0	0	0
Mutwanga	15	3	20.0%				0
Oicha	29	16	55.2%				0
Vuhovi	32	10	31.3%				0
Upper Uele	352	146	41.5%	3	1	1	2
Boma	41	17	41.5%				0
Mangbetu	1	1	100.0%				0
Dungu	3	1	33.3%				0
Gombari	102	44	43.1%				0
Pawa	87	40	46.0%	0	0	1	1
Rungu	4	1	25.0%	1	0	0	0
Wamba	114	42	36.8%	2	1		1
Tshopo	48	16	33.3%	2	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	27	10	37.0%	2	0	0	0
Mangobo	6	3	50.0%				0
Tshopo	1	0	0.0%				0
Wanie - Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	10	4	40.0%	0	0	0	0

Buta	1	1	100.0%				0
Viadana	6	1	16.7%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	0	0	0	0
Bulu	2	1	50.0%				0
Total	8442	4080	48.3%	66	22	16	38

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for October 2, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	23	2	25	21	2	2	0	23	10
Bas Uélé	153	3	156	18	2	135	1	20	0
Ituri	1089	75	1164	129	75	959	0	204	111
North Kivu	937	39	976	116	31	829	0	147	82
South Ubangi	57	0	57	1	0	56	0	1	1
Tshopo	111	4	115	5	5	105	0	10	3
Total General	2,370,123	2,493,290			115 2 086	1		405	207

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

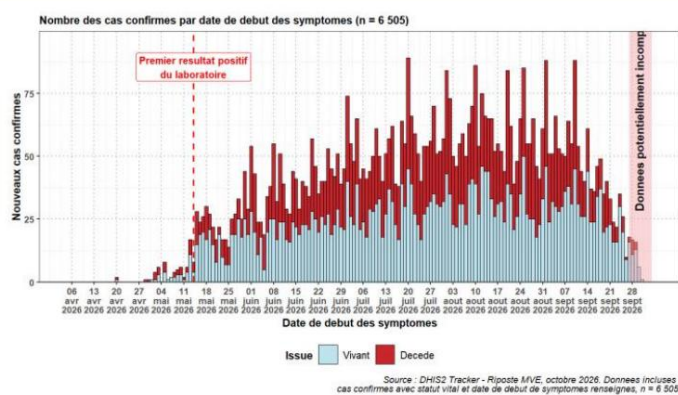
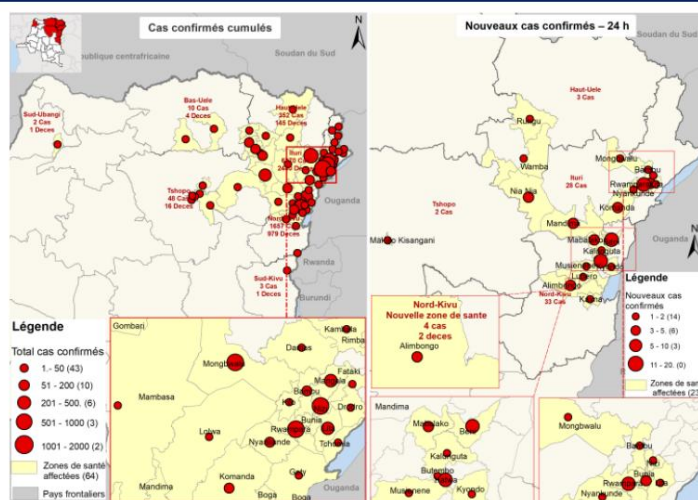


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Support for response activities in the affected provinces through the technical pillars.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

For October 2nd, **2,493 alerts** were reported, and **2,492** (99.9%) were verified. of which **405** (16.3%) were validated as suspected cases. Of these, **405** (100%) were investigated. Thus, 207 suspected cases were transferred to CT/CTE (Table 3).

Of the **32,654** contacts currently being monitored, **23,876** were seen in the last 24 hours, representing a follow-up rate of **73.1%**. This is lower than the previous day's rate (73.1%) but remains below the required threshold (95.0%). This rate was 94.6% (175/185) in Bas-Uélé, 87.0% (1,631/1,874) in Haut-Uélé, 86.0% (583/678) in Tshopo, and 82.9% (12,660/1,527) in [unspecified location]. Ituri, 77.3% (34/44) in South Ubangi, 60.2% (8,793/14,595) in North Kivu.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Eight (8) alerts were notified to the PoE/PoCs listed in Table 4, seven (7) in Ituri and one (1) in North Kivu. In Ituri, all seven alerts were validated: two suspects from the Mudzibala PoC (Bunia Health Zone) were transferred to the Elihya and CME Treatment Centers, the suspect from the Nizi DGRPI PoC was referred to the Nizi General Referral Hospital, and at the Mabakese and Mankoko 2 PoCs (Mambasa Health Zones), three suspects were transferred to the Mambasa Treatment Center and the fourth refused (transfer rate of 85.7%). In North Kivu, the alert from the Mukulia PoC (Beni Health Zone) was invalidated. In Bas-Uélé, outside of Table 4, all seven (7) alerts from the Sukisa and Koteli PoCs were invalidated (national total: 15).

Table 4. Status of key indicators for PoE/PoC by province, as of October 1, 2026

Indicators	Ituri	North Kivu	Tshopo	Overall
Report completeness	96.7% (58/60)	81.0% (17/21)	100.0% (18/18)	93.9% (93/99)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	147,336	98,712	22,441	268,489
% of travelers screened	95.9%	98.2%	99.2%	97.0%
% of travelers who washed their hands	97.5%	98.2%	98.7%	97.9%
% of travelers made aware	99.5%	98.2%	100.0%	99.0%
Number of alerts notified	7	1	0	8
Number of validated live alerts (suspected cases)	7	0	0	7
Number of bodies recovered today	0	0	0	0
% of suspected cases transferred to CTE/CT	85.7%	N / A	N / A	85.7%
% of lifeless bodies swabbed/secured	N / A	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	100.0%	N / A	N / A	100.0%

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 28 new positive results** (23 living and 5 dead) out of 171 samples received and analyzed (**positivity: 16.4%**).
- **North Kivu: 33 new positive results** (17 living and 16 dead) out of 116 samples received and analyzed (**positivity: 28.4%**).
- **Haut-Uélé: 3 new positive results** (2 living and 1 death) out of 20 samples received and analyzed (**positivity: 15.0%**).
- **Tshopo: 2 new positive results received** (2 live) out of 9 samples received and analyzed (positivity: 22.2%).
- **Bas Uélé: 7 samples** (3 blood and 4 swabs) received and analyzed at the Buta laboratory, **all returned negative**.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **Ituri:** 21 out of 39 rings were opened (53.8%), 70 out of 76 households were decontaminated (92.1%), and 12 out of 13 health facilities were decontaminated (92.3%). Of 69 EDS alerts (including 8 postponements), 49 EDS were carried out (71.0%) and 38 bodies were swabed.
- **North Kivu:** 15 out of 25 rings were opened (60.0%), 7 out of 30 households were decontaminated (23.3%), and 15 out of 20 ESSs (75.0%) were decontaminated; no decontaminated household received a kit, and 4 IPC scorecard assessments conducted in Butembo yielded an average score of 42%. Out of 51 EDS alerts, 51 EDSs were carried out (100%), and 51 bodies were swabed.
- **Haut-Uélé :** 3 out of 4 rings were opened (75.0%), 25 out of 29 households were decontaminated (86.2%), and 3 out of 3 ESSs were decontaminated (100%); 15 ESSs were supervised, 64 service providers were briefed (100%), and the Pawa, Isiro, and Wamba general hospitals received IPC kits. Out of 7 EDS alerts, 6 EDSs were carried out (85.7%), and 6 bodies were swabed, with one refusal in Isiro (Tuluba health center).
- **Tshopo :** 18 out of 18 social enterprises were monitored in 6 health zones and 186 service providers were briefed (100%); the Integrated training of 150 private healthcare professionals (PHPs) from Makiso-Kisangani has been completed, and a church, a reflexology center (cases 45), and the Kawaki clinic (cases 44) have been decontaminated. Cumulatively, all 35 healthcare facilities (100%) and 26 out of 31 households (83.9%) that hosted a confirmed case were decontaminated within 48 hours.
- **Bas-Uélé :** 10 out of 10 ESS (100%) were monitored in Viadana, Buta and Ganga, 54 out of 54 providers (100%) and 10 hygienists were briefed and 3 out of 3 triages (100%) are functional in Buta. Of 3 death alerts (2 in Ganga and 1 in Buta), 2 bodies were swabed in Ganga, without EDS carried out, and one failure was recorded in AS Triangle (ZS of Buta).

2.4.2. Challenges

In **Ituri**, the opening of the rings remains insufficient (53.8%; 21/39) due to a lack of teams in Komanda, Mangala, and Mandima, and a strike by teams in Bunia; 10 bodies were not swabed due to a lack of supplies and teams in Mandima or due to refusal by families; resistance persists in Bambu, Rwampara, Mangala, and Fataki; EDS teams lack transportation in Kilo, Nia-Nia, Mandima, Damas, and Fataki, and no households, health establishments, or schools have received kits. In **North Kivu**, household decontamination remains low (23.3%; 7/30), no households have received kits, and the average IPC score of the health establishments assessed in Butembo reaches only 42%. In **Haut-Uélé**, only 4 out of 7 health zones report their activities, the health zones of Pawa and Wamba do not have a standardized CTE, the population of the Gbonzunzu AS (Pawa) remains hostile and only 2 out of 15 ESS reach 80% on the PCI score).

2.5. Holistic care

2.5.1. Main actions

- In **Ituri**, 74 new admissions were recorded compared to 90 discharges, including 9 recoveries. A total of 378 patients remained in isolation at the end of the day. The bed occupancy rate was **37.2%** ; 336 out of 359 patients (93.6%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (13/13), in daycare (5/5), and in the community (50/50); 31 patients are in critical condition.
- In **North Kivu**, 79 new admissions were recorded, compared to 98 discharges (14 recoveries). A total of 364 patients remained hospitalized, including 261 in standardized facilities. The bed occupancy rate was **63.0%** ; 21 patients were in critical condition at the Beni Ebola Treatment Center.
- In **Tshopo**, 3 new admissions were recorded. Compared to 1 discharge, 14 patients remained in isolation at the end of the day, representing an occupancy rate of 45.2%.
- In **Bas-Uélé**, 16 suspected patients were in isolation at the end of the day in the CTs of Poko (8), Bondo (5), Limba (ZS of Viadana, 2) and Koteli (1); a release of a suspected case declared not a case was recorded in Buta.
- In **Haut-Uélé**, 11 new admissions were recorded. In total, 74 patients are in isolation in CTEs; against 14 discharges, including 8 recoveries (1 in Isiro and 7 in Wamba), 5 non-cases and 1 confirmed death in Paw, representing an occupancy rate of **53.6 %**.

2.5.2. Challenges

- In **Ituri**, insufficient capacity of some CTE/CT facilities and inadequate care food-related issues remain a concern.
- In **Bas-Uélé**, the lack of a standardized CTE and ambulance for the transfer of patients remains a major operational challenge.
- In **Tshopo**, the continued presence of confirmed cases in isolation necessitates the maintenance of infection prevention and control measures.

2.6. Vaccination

2.6.1. Main actions

- In **Bas-Uélé**, vaccination of PPL and TPL continued in the health zones of Buta and Aketi and compassionate vaccination started in the health zone of Viadana (13 vaccinated), bringing the provincial total to 1,147 people vaccinated out of a target of 13,202 (8.7%), mainly in Buta (613; 21.4%) and in Ganga (273; 20.5%).
- In **Ituri**, vaccination of PPL and the study on Ervebo/Bundibugyo cross-immunity were evaluated with the support of MSF.

2.6.2. Challenges

In **Ituri**, the target for PPL/TPL remains to be redefined, which prevents the calculation of a reliable coverage rate.

In **Bas-Uélé**, the UCCs have not been put into operation, the validated micro-plan and vaccination funds are not available.

2.7. Continuity of Care

2.7.1. Main actions

- In **Ituri**, the capacities of ECZ members, IT, ITA and community health workers were strengthened on community-based surveillance and care at SSC in the health zones of Nizi, Mongbwalu, Nyankunde and Rwampara, and the budget allocation tools for the health zones were analyzed for the free healthcare project.
- In **Bas-Uélé**, the Ganga/Dingila health zone was evaluated regarding HIV screening and ART initiation new cases, by comparing the periods from May to August 2025 and 2026.
- In **Tshopo**, the evaluation of the Wandungu Health Center (Kabondo Health Zone) found 9 out of 11 staff vaccinated (81.8%), 8 pregnant women absent out of 25 expected for prenatal care (32.0%), the absence of a triage area and poor waste management; the recovery plan for the Zawadi Health Center was developed.
- In **Haut-Uélé**, the evaluation of the Tuluba health center (Isiro health zone) and Maman Kayenga health center (Wamba health zone) showed a sharp decline in curative consultations (35 in August compared to 84 in January), antenatal care (77 compared to 131) and deliveries (30 compared to 55); the Boma Mangbetu, Pawa and Wamba general hospitals received IPC kits from UNICEF.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- In **Ituri**, 29,770 home visits reached 106,322 people; 1,278 educational talks reached 17,109 people, 2 community debates reached 191 people, and 83 mass awareness campaigns reached 3,291 people. A total of 389 community alerts, including 106 deaths, were reported; 244 out of 256 feedback requests (95.3%) were resolved within 48 hours, and no incidents were reported (9 out of 28 health zones completed, or 32.1%); the CREC facilitated the pre-listing of 394 contacts around 12 confirmed cases and 13 out of 13 EDS (Emergency Health Diagnosis).
- In **North Kivu**, 11,012 home visits reached 35,130 people, 156 educational talks reached 5,328 people, and 36 mass communication sessions reached 6,696 people; 466 out of 650 expected community alerts (71.7%) were reported, and 24 out of 30 feedback requests (80.0%), as well as the incident recorded in Katwa (refusal of decontamination), were resolved within 48 hours. The local security council approved the construction of the Mabalako Ebola Treatment Center in Mangina.
- In **Tshopo**, 207 home visits reached 1,186 people, and 3 educational talks reached 224 people and 7 mass awareness campaigns reached 3,958 people; the mayors of the 6 communes and local leaders were engaged around the Mayor of Kisangani, 13 community alerts were notified (8 in Bafwasende, 3 in Kabondo and 2 in Tshopo) and the 3 identified refusals were lifted.

- In **Bas-Uélé**, 80 home visits reached 770 people, and 3 educational talks reached 100 people and 2 mass awareness campaigns 570 people in Viadana, Titule and Ganga, including in schools at the Maman Sinovia Institute (Ganga Health Zone); the community alert notified in Ganga has been dealt with (1/1).
- In **Haut-Uélé**, 394 home visits reached 1,493 people, 152 educational talks reached 634 people and 2 mass awareness campaigns reached 369 people, and an interactive program was organized on RTK radio; 22 community alerts were notified in 5 health zones, and the 3 feedbacks as well as the recorded incident (refusal lifted in AS Tely, Isiro health zone) were resolved within 48 hours.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- In **North Kivu**, 25 confirmed cases out of 49 (51.0%) from the previous day, 99 confirmed cases under follow-up out of 135 (73.3%), 37 new suspected cases out of 73 (50.7%), and 93 suspected cases under follow-up out of 144 (64.6%) benefited from SMSPS interventions. The teams supported 256 caregivers and 103 people living with HIV (PLHIV), monitored 23 recoveries out of 27 (85.2%) in the community, and announced 78 results, including 25 positive ones; two self-care sessions brought together 75 PLHIV at the Rwampara Ebola Treatment Center (ETC).
- In **Ituri**, 34 out of 44 confirmed cases (77.3%) from the previous day, 126 confirmed cases under observation out of 133 (94.7%), 57 new suspected cases out of 100 (57.0%), and 122 suspected cases under observation out of 133 (91.7%) benefited from SMSPS interventions. The teams supported 447 caregivers and 78 people living with HIV and reported 56 results, including 34 positive ones.
- In **Tshopo**, the teams provided psychological support to the 3 confirmed cases followed at the CTE Météo (100%), carried out psychoeducation for 8 suspects and 1,276 people, including 1,177 students in Lubunga and Bafwasende, and lifted 3 out of 4 resistances; 5 out of 7 health zones that notified are covered (71.4%).
- In **Haut-Uélé**, 3 new confirmed cases, 22 confirmed cases under follow-up, 7 new suspected cases and 43 suspected cases under follow-up benefited from SMSPS interventions; the teams supported 49 accompanying persons, 57 PPLs and 14 children separated in the community and announced 28 results, including 7 positive ones.

2.9.2. Challenges

- In **Ituri**, only 13 out of 28 affected health zones are covered (46.4%) and the capacities of Psychologists and APS/TPS must be reinforced in the Mandima health zone to ensure interventions around cases.
- In **Haut-Uélé**, the APS (Agents of Security) of Wamba and Boma Mangbetu lack motorcycles, kits
Nutritional resources for weaned infants are insufficient in Wamba and Isiro, and data reporting remains low in Boma Mangbetu and Pawa.
- In **Tshopo**, there is a need to train the APS (Auxiliary Security Personnel) of the ZS (Health Zones) of Wanie-Rukula and Bafwasende, and insufficient means of communication and movement of the pillar.

2.10. Logistics

2.10.1. Main actions

- In **Bas-Uélé**, deployment of surveillance teams in Ganga and Viadana, inventory of DPS inputs and continuation of the rehabilitation of the DPS offices (WHO) and the construction of the transit center of the HGR of Buta.
- In **Tshopo**, allocation of the 2 rental vehicles provided by MOMENTUM, reception of 64 800 syringes (WHO) and distribution of 57 DHIS2 Tracker tablets to IT departments in Makiso-Kisangani, Tshopo and Kabondo; construction of the Kongakonga isolation facility, the Bafwasende CTE and the extension of the Météo CTE continue.

2.11. Security

2.11.1. Main actions

2.12. PSEA (Prevention of Sexual Exploitation and Abuse)

2.12.1. Main actions

- **In Ituri**, 49 staff members (27 men and 22 women) were briefed on the PSEAH and signed the code of conduct, including 30 focal points from five ZS trained in Bunia and 19 providers from the CTE in Lita; 300 people were sensitized in Bunia, Rwampara and Komanda and only 13 out of 36 ZS (36.1%) know the reporting mechanism.
- **In North Kivu**, 38 service providers (24 men and 14 women) were briefed on the PSEAH in Beni and 69 people were sensitized; the Head of the PSEA pillar of the National Coordination brought together all the PSEA stakeholders of the response; 6 out of 34 health zones (17.6%) know the reporting mechanism.
- **In Haut-Uélé**, 6 PPLs from the Isiro General Referral Hospital were briefed on the PSEAH and 63 people, mainly women receiving prenatal care at the Tely Health Center and the General Referral Hospital, were sensitized during 4 sessions; 6 out of 13 health zones (46.2%) know the reporting mechanism,
- **In South Kivu**, 64 IMC staff and partners were briefed on the PSEAH in the health zones of Miti-Murhesa and Katana; 18 out of 34 health zones (52.9%) know the reporting mechanism.

2.12.2. Challenges

At the national level, only 2 DPS reported their PSEA activities (57.1%) and CBCMs remain largely ineffective.

KEY MESSAGES

- Active and sustained community transmission remains observed in the provinces of Ituri, North Kivu, and Haut-Uélé. Meanwhile, the province of Sud-Ubangi has not reported any confirmed cases in the last 24 hours, and a lull has been observed in the province of Tshopo compared to the last 72 hours.
- Progress has been made in the field of epidemiological investigations, but it needs to be consolidated: all (100 %) of the
All suspected cases were investigated within 24 hours of notification. However, the contact tracing rate is lower than the previous day (73.1% vs. 80.3%). Efforts must be made to bring this rate up to the required threshold (95%).
- Strengthen surveillance, care, IPC, logistics and community engagement, prioritizing increased capacity of CTE/CT in North Kivu, rapid transfer of suspects and isolation of confirmed cases.
- Strengthen preparedness and response to Ebola Virus Disease (EVD) in the river corridor and surrounding areas, particularly in South Ubangi, through active case finding, contact tracing, and ensuring the availability of supplies. Continue cross-border surveillance with South Sudan.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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